

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Melatonin 2mg prolonged-release tablets (Circadin) / Melatonin 1mg, 2mg, 3mg, 5mg tablets for the treatment of Primary Insomnia

Summary of NICE / NICE CKS Guidelines for Primary Insomnia

Overview

Insomnia is persistent difficulty initiating and/or maintaining sleep. It is common in adults and significantly affects quality of life. Short-term insomnia is often situational, but chronic insomnia can have significant health impacts.

Assessment

- **Sleep history:** Duration of insomnia, patterns of sleep disturbance (difficulty initiating, maintaining sleep, or early morning awakening).
- **Sleep hygiene evaluation:** Review of sleep environment, routine, caffeine/alcohol use, screen time before bed.
- **Underlying causes:** Screen for depression, anxiety, pain, sleep apnoea, medications, shift work, other medical conditions.
- **Severity assessment:** Impact on daytime function, mood, work, relationships.

Management

- **First-line:** Sleep hygiene education, cognitive behavioural therapy for insomnia (CBT-i), consideration of lifestyle factors.
- **Pharmacotherapy:** Melatonin for short-term use in adults aged 55 and over (Circadin is NICE-approved for this indication). Off-label use in younger adults may be considered after sleep hygiene and CBT-i attempted.
- **Review:** Regular assessment of need; medications should not be used as long-term solution without periodic review and reassessment.

Red flags requiring onward referral

- Suspected sleep apnoea (witnessed apnoeas, excessive daytime sleepiness, loud snoring).
- Parasomnias (sleepwalking, night terrors, REM sleep behaviour disorder).
- Suspected narcolepsy or other primary sleep disorder.
- Severe psychiatric illness requiring specialist assessment.
- Insomnia secondary to uncontrolled medical condition.

Patient Group Direction

for the supply/administration of

Melatonin 2mg prolonged-release tablets (Circadin)

for the treatment of

Primary Insomnia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Short-term treatment of primary insomnia in adults characterised by poor quality sleep. Melatonin is indicated for short-term management of insomnia in patients aged 18 years and over.
Inclusion criteria	Adults aged 18 years and over. Diagnosed with primary insomnia (persistent difficulty initiating and/or maintaining sleep for a minimum of 4 weeks). Sleep hygiene measures have been attempted and documented. Informed consent obtained; patient understands that melatonin is for short-term use only. No contraindications identified.
Exclusion criteria	Known hypersensitivity to melatonin or any excipients. Autoimmune disease (relative contraindication; melatonin may stimulate immune function). Severe hepatic impairment (Child-Pugh Grade C). Concurrent use of fluvoxamine (significantly increases melatonin

	levels; major interaction). Pregnancy or breastfeeding. Galactose intolerance, lactase deficiency, or glucose-galactose malabsorption (Circadin contains lactose).
Cautions	Hepatic impairment (mild to moderate). Renal impairment. Autoimmune conditions. Concurrent CYP1A2 inhibitors (ciprofloxacin, oestrogens, fluvoxamine) - melatonin levels may be elevated. May cause drowsiness; caution with activities requiring alertness (driving, operating machinery). Diabetes mellitus (melatonin may affect glucose tolerance). Epilepsy or history of seizures.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Melatonin 2mg prolonged-release tablets (Circadin)
Legal category	POM (Prescription Only Medicine)
Storage	Store below 25°C. Protect from light. Keep in original container.
Route/method of administration	Oral. Circadin: swallow whole tablet with water, 1-2 hours before bedtime. Do not crush, chew, or divide.
Dose and frequency	Circadin (prolonged-release): 2mg once daily, taken 1-2 hours before bedtime. Swallow tablet whole with water; do not crush, chew, or divide. Immediate-release melatonin tablets: 1-5mg once daily, taken 1-2 hours before bedtime (dose varies by preparation). Individual dose titration may be required; start with lower dose in elderly or renally impaired patients.
Quantity to be administered and/or supplied	Up to 28 tablets (equivalent to 4-week supply for once-daily dosing).
Maximum or minimum treatment period	Up to 13 weeks initially for Circadin. Review clinically at 4 weeks; continue only if beneficial. After initial 13-week period, consider intermittent use pattern. Melatonin is not recommended for long-term continuous use without periodic specialist review and reassessment of need.

Adverse effects	Common: Headache, nasopharyngitis, back pain, arthralgia. Uncommon: Dizziness, somnolence, irritability, abnormal dreams, anxiety, migraine, dry mouth, abdominal pain. Rare: Disorientation, mood alteration (depression, mood swings), aggression. Very rare: Thromboembolism (disputed causality).
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with melatonin. Advise patient on sleep hygiene measures and expectations.
Follow-up advice to be given to patient or carer	Take the tablet 1-2 hours before bedtime; swallow whole with water. Maintain good sleep hygiene: avoid screens for 1 hour before bed, keep bedroom cool and dark, maintain regular sleep schedule. Avoid caffeine, alcohol, and heavy meals close to bedtime. Melatonin is for short-term use only; do not exceed recommended duration without review. May cause drowsiness the next morning; avoid driving or operating machinery if affected. Contact GP or healthcare professional if insomnia persists after 4 weeks of treatment, or if new sleep problems develop.

	Report any mood changes, anxiety, or unusual dreams. Do not share medication with others.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Insomnia
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		
Name of professional group signatory	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Melatonin 1mg, 3mg, 5mg tablets (unlicensed/off-label)

for the treatment of

Primary Insomnia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Short-term treatment of primary insomnia in adults. Off-label use in patients under 55 years, or as alternatives to Circadin.
Inclusion criteria	Adults aged 18 years and over with primary insomnia (≥4 weeks duration). Sleep hygiene measures attempted. Patient counselled regarding unlicensed status if applicable. Informed consent obtained.
Exclusion criteria	Known hypersensitivity to melatonin or excipients. Autoimmune disease (relative). Severe hepatic impairment.

	Concurrent fluvoxamine. Pregnancy or breastfeeding. Lactose intolerance (if tablet contains lactose).
Cautions	Hepatic impairment (mild to moderate). Renal impairment. Autoimmune conditions. CYP1A2 inhibitors. Drowsiness; caution with driving. Diabetes (glucose tolerance effects). Epilepsy.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Melatonin 1mg, 3mg, 5mg tablets (unlicensed preparations)
Legal category	POM (Prescription Only Medicine) - unlicensed melatonin is POM in the UK
Storage	Store below 25°C, protect from light. Keep in original container.
Route/method of administration	Oral. Swallow whole tablet with water, 1-2 hours before bedtime.
Dose and frequency	Melatonin 1-5mg once daily, taken 1-2 hours before bedtime. Typical doses: 1mg for sensitive patients; 2-3mg for general use; 5mg for higher doses. Tablets should be swallowed whole; some may be dispersible.
Quantity to be administered and/or supplied	Up to 28 tablets (4-week supply).
Maximum or minimum treatment period	Up to 13 weeks initially, with 4-week clinical review. Not recommended for long-term use without periodic reassessment.
Adverse effects	Common: Headache, nasopharyngitis, back pain, arthralgia. Uncommon: Dizziness, somnolence, irritability, abnormal dreams, anxiety, migraine, dry mouth, abdominal pain. Rare: Disorientation, mood alteration, aggression.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk
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	and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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